Medallia

June 16, 2025

The Honorable Mehmet Oz
Administrator
U.S. Department of Health and Human Services (HHS)
Centers for Medicare & Medicaid Services (CMS)

Steven Posnack, MS, MHS
Principal Deputy Assistant Secretary for Technology Policy and
Principal Deputy National Coordinator for Health Information Technology
U.S. Department of Health and Human Services
Assistant Secretary for Technology Policy and Office of the National Coordinator for Health IT (ASTP/ONC)

Submitted electronically at regulations.gov
Docket ID: CMS-2025-0050

Dear Dr. Oz and Mr. Posnack,

Subject: Request for Information (RFI); Health Technology Ecosystem [CMS-0042-NC]

On behalf of Medallia, we are sincerely thankful for the opportunity to participate and submit the following comments to this RFI for question Digital Health Adoption/VB-2. Founded in 2000 as an entirely US-owned company, Medallia is a Software as a Service (SaaS) Customer and Employee Experience Technology platform that holds a FedRAMP High Authorization allowing it to process the most sensitive data and workloads across the Federal Government. The platform powers agencies including the Department of Veterans Affairs, Office of Personnel Management, Department of Homeland Security, General Services Administration, Federal Student Aid, US Department of Agriculture, and the Inland Revenue Service amongst others.

We appreciate your continued efforts to promote innovation and engage stakeholders in shaping future rulemaking on policies that advance the digital health ecosystem. Please do not hesitate to contact me directly with any questions.

Respectfully submitted,

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Medallia's response to Digital Health AdoptionVB–2. How can key themes and technologies such as artificial intelligence (AI), population health analytics, risk stratification, care coordination, usability, quality measurement, and patient engagement be better integrated into APM requirements?

Overview

To deliver on its mandate to modernize Medicare, reduce waste, and improve patient outcomes, CMS must ground innovation and operations/program execution in real-time patient and beneficiary experience data - ideally using AI and technology that can integrate and analyze feedback and experience data with meta-data and operational systems, and route those relevant insights for action (not just raw data) across CMS for agency wide improvements.

Similar to the practices and technology stack that the leading Fortune 500's - including healthcare Payers use - by capturing, routing, and surfacing voice-of-the-customer data from Medicare Contact Centers (eg. 1-800-Medicare and web chats) or consolidated call center efforts, Healthcare.gov, and other digital or journey touchpoints (most of which exist at CMS but are siloed), CMS can **be pro-active** in driving more targeted improvements, reduce operational waste, and elevate cross-agency performance.

Dr. Oz shared his Medicare enrollment experience and tips navigating with the public last week. As he looks to improve that experience for the millions of eligible Americans and others looking for a better healthcare journey, imagine if he, and his entire team across CMS could receive continuous and personalized insights - **understanding what actions to take that are most effective for innovation/enrollment and to more effectively drive down costs - across different patient journeys and omni-channel programs.** This could be to answer questions, improve, or get notified on a specific contact center team who are not performing well, actions to reduce costs within or across Medicare Administrative Contractors (MAC), nuances of what issues are specific to a jurisdiction or State along the medicare journey, alerts on specific topics like fraud/waste/abuse, digital products not working, repeat calling, etc.), or how effective a product or healthcare innovation launch is going?

There are millions of experience data points existing within CMS currently and continuously increasing **via its contact centers** - specifically in the National Data

Warehouse (NDW) and via digital/meta-data collected through other disparate systems- that is siloed and held within a few programs/groups and contractors.

Moreover, the capacity to automate not only the insights from the calls to improve call center operations (e.g. reduce call time, improve first call resolution, improve agent retention - which amounts to tens to hundreds of millions of dollars saved each year) most contracts and programs running the contact centers are not leverage experience data in a way that is streamlining operations, saving money or reducing overhead.

As we have seen from the best and most complex companies and agencies, access to and insights from customer experience data is not optional—it is mission-critical. And using technology that can integrate, analyze at scale, and route dynamically in near-real time so the whole of the CMS has relevant data to take the most effective and efficient action. Without it, CMS risks designing systems in the dark, misallocating funds, and leaving people behind, especially the most vulnerable. Data-informed CX is essential to making Medicare more accessible, equitable, and trusted.

Medallia Chief Strategy Officer Sid Banerjee digs into this in the following clip: https://exp-now.medallia.com/video/experience-25-keynote-sid/

Current Challenges and Risk for CMS

The amazing plans that CMS has for major innovation, reducing waste, and drastically saving costs and its stakeholders - will always be at risk of program failures, enrollment challenges, rising servicing costs, and inefficient actions due if they do not have true insights from the voice of the people and lack of ability to democratize insights across the agency (which leads to lagging decision/improvements or rapid actions that cause major problems or costs due to a siloed or nonexistent risk mitigation).

1. Reduced Enrollment Effectiveness

- Ineffective outreach and communication: Without insights into what information confuses or engages customers, CMS may continue to produce materials or digital experiences that are difficult to understand or poorly targeted.
- Missed enrollment opportunities: Without real-time data on drop-off points in the enrollment funnel, CMS cannot identify where or why users abandon the process, resulting in fewer enrollments.

2. Poor Customer Experience and Satisfaction

- Increased frustration: Without feedback from touchpoints (calls, web sessions, chatbot use, etc.), CMS may be unaware of usability issues or barriers—such as confusing eligibility steps, long hold times, or inaccessible digital platforms.
- Trust erosion: Negative experiences that go unaddressed can erode public trust in CMS and government healthcare programs.

3. Government Blind Spots in Health Access

- One-size-fits-all from Washington: Without clear, localized data, federal agencies like CMS risk creating bloated, top-down programs that overlook the real needs of seniors and working Americans—especially in rural, low-income, or non-English-speaking communities.
- Missed voices, wasted dollars: When bureaucracy ignores input from real people—such as the disabled, immigrants, or those navigating the system without help—policies end up wasting taxpayer money on generic solutions that don't work on the ground.

4. Operational Inefficiencies and Waste

- Misallocation of resources: Without knowing which aspects of the enrollment journey work or fail, CMS may overinvest in ineffective tools or underinvest in high-impact services like live support or culturally competent outreach.
- Duplicative efforts: Lack of CX data can lead to siloed initiatives that don't coordinate or learn from each other, increasing administrative burden.

5. Stagnation in Innovation

- Limited data for policy pilots: Innovation requires evidence. Without robust user journey data, CMS cannot evaluate or scale pilot programs (e.g., AI-driven chatbots, simplified applications, or personalized plan recommendations).
- Slow responsiveness to change: CMS may not detect emerging user needs or behavior shifts in time to adapt, such as increased mobile use or shifting perceptions of Medicare Advantage.

6. Compliance and Oversight Gaps

- Failure to meet federal mandates: Agencies are increasingly required to measure and improve customer experience (e.g., Executive Order 14058 on Transforming Federal Customer Experience). Lack of CX data puts CMS at risk of non-compliance.

- Inability to demonstrate value: Without data showing improved experiences and outcomes, CMS may face difficulty justifying budgets or policy proposals to stakeholders, including Congress.

Solution

By leveraging a FedRamp High CX platform (with Speech Analytics that connects directly with Digital feedback/insights capabilities and AI that routes insights across the organization from top-bottom) CMS can rapidly convert voice-of-customer data (e.g. 100% visibility of calls, digital behavior,) into agency-wide action—unlocking: more accurate understanding of progress in healthcare innovation/pilots/new programs, patient engagement and issues in enrollment, and operational efficiency and operational waste.

Leveraging the audio-files and associated meta data from calls from 1-800-Medicare (located in the National Data Warehouse) can enable faster, more transparent routing of feedback from call centers, HealthCare.gov, and post-service surveys directly to the correct CMS teams and systems, Medallia allows CMS to:

- Improve patient engagement by acting on what matters most to beneficiaries—reducing friction, confusion, and frustration;
- Enhance usability of CMS digital services by identifying the highest-impact improvements across self-service journeys;
- Advance risk stratification by surfacing hidden signals of need or vulnerability across demographic and regional lines;
- Accelerate care coordination by giving all departments and contractors access to real-time experience trends and sentiment;
- Reduce costs and save time across all divisions—from policy to 100% transparency across contact centers (including MAC oversight) to digital operations.

***NOTE - we have left out specific confidential and proprietary information on the solution section and will email a separate document.**

We recommend using a Fedramp High Customer Experience tool (w/speech analytics capabilities) that can integrate seamlessly with CMS's current and future tech

stacks—including Salesforce, ServiceNow, Oracle, AWS, and AI/data platforms—making innovation more scalable, efficient, and future-ready. This strategy aligns directly with the Trump/Oz administration's goals of reducing government waste, modernizing Medicare, and running CMS with private-sector efficiency.

Alignment with Innovation and Accountability

Focus Areas	What a FedRamp High CX Solution Offers
Smarter Government	Data-informed improvements replace trial-and-error with targeted modernization
Cost Savings and Efficiency	Reduces waste from misaligned programs, excess call volume, and failed pilots
Medicare Modernization	Transforms CMS into a responsive, tech-enabled organization
Cross-Agency Innovation	Feedback routes across departments in real-time, eliminating bureaucracy
Tech-Forward Public Service	FedRamp High Solution integrations future-proof CMS's innovation infrastructure

Contact Center and Quality Assurance Specific Solutions and Benefits

Example from a current agency using this method and technology for multi-million dollar cost savings and time-savings

A current Federal agency has used audio files and meta-data (call records located in their cloud) to not only realize the program, policity, and digital communication benefits across multiple program and cross-sector objectives, but also better manage its contact centers and possible consolidation.

Through this approach this agency has been able to set up a scalable solution to providing vendor oversight of its most expensive line items – contact centers and servicing. With the implementation of the Medallia Speech Tool, there are four areas where this solution is making it possible to achieve these outcomes. Without this type of tool, these savings and improved customer experience would not be possible at scale.

1. **Self-service Drivers**
   An agent handling a phone call from a customer is the most expensive way a contact center can provide service. Agency has developed a few self-serve options but not all were effectively promoted or used, and others could be added. These efforts and data associated are also siloed amongst vendors. Using the Speech and Voice Analytics tool, Agency is able to pinpoint how many customers are calling when they could have used an existing self-serve option. Armed with this information, they will be able to assertively guide and direct more of their callers to use self-serve options and thereby reduce the Agency's overall call volume (across its many contact centers). Private sector companies use this to great effect and have experienced anywhere from 8% to 20% conversion of callers to self-service users.

2. **One-call Resolution Rate**
   While the Agency does not publicly publish a one-call Resolution benchmark, best-in-class industry standards for our type of business ranges from 70% - 85%. Given this Agency's high abandon rate it is estimated that it could be anywhere between 40% to 55% meaning as much as 60% of this Agency's call volume could be from repeat callers who have not been able to reach an agent or have gotten inaccurate information when they did reach one. Reducing repeat callers has the potential to significantly reduce call volumes and dramatically cut costs – in the millions of dollars. Implementing this tool will make possible establishing a One-call Resolution Rate benchmark and provide the insight on ways to consistently improve it – without this type of tool it will not be possible.

3. **Efficiency Savings**
   Based on feedback from the private and government sectors, users of this technology have realized efficiency gains anywhere from 10% to 30%. Even using the low end of this scale allows this Agency to perform more in depth and significantly more effective QA without having to hire new personnel. These efficiencies will also position this Agency to control the growth of staffing as the customer base grows in the future.

4. **Cost Avoidance**

This Agency's only remotely feasible alternative to investing in this type of technical solution is to hire more people to achieve a fraction of the benefit. In determining the most viable paths forward, increasing staffing was studied and through it the Agency determined that it could potentially increase its sampling rate of monitored calls to 2-3% from a current baseline of 0.5% but only by hiring 11 new contractor staff that would cost ~$9 million a year. This annual cost will be avoided and hence saved with implementation of this tool while gaining the benefit of capturing and transcribing 100% of recorded calls without having to hire additional contractors or employees.

5. **Vendor Downsizing or Elimination**
   The implementation of this type of tool will arm this Agency with a tremendous amount of data and the AI that can be used to truly understand the volumes, the nature, and types of contacts enabling them to organize its contact center work more efficiently while maintaining effectiveness. With this tool, this Agency will be able to identify opportunities to streamline and automate several workflows where having 10+ contact centers may not be necessary and could possibly be reduced. This will greatly simplify this Agency's contractor footprint representing potentially significant cost savings to the organization.

IV. Recommendation: Invest in Unified, Intelligent Feedback Infrastructure

We recommend CMS adopt a FedRamp High Customer Experience solution (with speech analytics capabilities) as a central experience intelligence platform, integrated across departments and systems, to:

- Use advanced CX technology to analyze all audio files/meta-data from the National Data Warehouse, whereby it would automatically route and provide role-based insights for improvements (by the job to be done across CMS's divisions, leadership, call centers, digital offices, ombudsman, etc) - for large scale cost savings and innovation.
- Accelerate innovation cycles with voice-of-the-customer data;
- Reduce operational and contractor costs through targeted action;
- Route experience insights directly to accountable POCs for timely follow-up;
- Build a modern, interoperable technology foundation for future CMS programs.

This approach makes cost-effective innovation repeatable, measurable, and scalable—anchored in the real experiences of Medicare beneficiaries.

Recommended Next Steps

1. Define innovation and savings metrics (e.g., call deflection, issue resolution time, pilot success rates).
2. Launch a pilot using audio files / chat transcripts and meta-data from National Data Warehouse and/or in conjunction with digital feedback via a FedRamp High customer experience analytics tool
3. Enable real-time routing rules and dashboards tied to department roles and issue types for CMS leadership and core staff throughout CMS divisions (e.g. CMMI, OC, CCSQ, OCIO).

Case Studies for Other Agencies and Fortune 100's will be sent in the confidential version.